Basic Examina

Clinical Postings | 3rd Year MBBS

Hi Sammy!
Examination skills =
your clinical superpower!
Practice makes perfect!

Examination | Clinical Postings | 3rd Year | MBBS

What's Inside

- General Examination & Vitals p.2
- Eye Examination — Visual Acuity, Pupil, Fundoscopy p.3
- ENT Examination — Otoscopy, Rhinoscopy, Laryngoscopy p.4
- Exam Strategy & High Yield p.5

General Examination + Vitals

Always start with
general examination!
Don't skip vitals!

EXAMINATION APPROACH

Inspection -> Palpation -> Percussion -> Auscultation
(Always follow this order for every system!)

Vital Signs — Normal Values

Vital Sign	Normal Range	Key Points
Pulse Rate	60-100/min	Rate, rhythm, volume, character, vessel wall
Blood Pressure	120/80 mmHg	Both arms, supine & standing if needed
Temperature	98.6F / 37C	Oral, axillary (-1F), rectal (+1F)
Respiratory Rate	12-20/min	Count for full 60 sec, note pattern
SpO2	>95%	Pulse oximetry on finger, check peripheries

General Examination — Checklist

Parameter	What to Look For
Built & Nourishment	Thin/average/obese, BMI assessment
Pallor	Lower palpebral conjunctiva, nail beds, palms — anemia
Icterus	Sclera (best seen in daylight) — jaundice
Cyanosis	Central (tongue), Peripheral (nail beds) — hypoxia
Clubbing	Nail angle >180, Schamroth sign — lung/heart disease
Lymphadenopathy	Cervical, axillary, inguinal — size, consistency, tender
Edema	Pedal (pitting), sacral, periorbital — check bilaterality
JVP	45-degree position, right IJV, normal <3 cm above sternal angle

★ PICICLEJ for General Exam!
Pallor, Icterus, Cyanosis, Clubbing, Lymphadenopathy, Edema, JVP

8-point eye exam is
a MUST for clinicals!
Learn the sequence!

Visual Acuity Testing

- Snellen chart at 6 metres (20 feet) — test each eye separately
- Record as: distance/line read (e.g., 6/6 = normal, 6/60 = severe)
- If can't read top line: FC (finger counting), HM (hand movements), PL/PR
- Pinhole test: if vision improves = refractive error (not pathological)
- Near vision: Jaeger chart at 33 cm (reading distance)

Pupillary Examination

- Size: Normal 3-5mm, check symmetry (anisocoria = unequal)
- Shape: Round, regular (irregular = synechiae, trauma)
- Direct reflex: Light in eye -> same eye constricts
- Consensual reflex: Light in eye -> opposite eye constricts
- RAPD (Marcus Gunn pupil): Swinging flashlight test — afferent defect

Slit Lamp + Fundoscopy

Examination	Key Points
Slit Lamp	Anterior segment: cornea, AC depth, iris, lens, IOP
Gonioscopy	Angle assessment (open vs closed angle glaucoma)
Direct Ophthal.	Red reflex, optic disc, cup:disc ratio, vessels, macula
Indirect Ophthal.	Peripheral retina, wider field, used with 20D/28D lens
Tonometry	IOP measurement: normal 10-21 mmHg (Goldmann = gold std)

Other Eye Tests

- Colour vision: Ishihara plates (screen for red-green deficiency)
- Visual fields: Confrontation test (quick) or Perimetry (detailed)
- Cover test: Detect squint/strabismus — cover one eye, observe other

Ear, Nose, Throat —
3 exams in 1 posting!
Know the instruments!

Ear Examination — Otoscopy

- Instrument: Otoscope (battery-powered with magnifying lens)
- Technique: Pull pinna up, back & laterally in adults (down in children)
- Look for: TM integrity, cone of light (5 o'clock in right ear), perforation
- Tuning fork: Rinne test (AC vs BC) & Weber test (lateralization)
- Normal Rinne: AC > BC (positive). Negative Rinne = conductive loss

Tuning Fork Tests — THE Comparison

Test	Technique	Conductive Loss	Sensorineural Loss
Rinne	Mastoid then EAC	BC > AC (negative)	AC > BC (positive)
Weber	Vertex of skull	Lateralizes to affected	Lateralizes to normal
ABC	Absolute Bone Cond	Normal	Reduced/absent

Nose Examination — Rhinoscopy

- Anterior rhinoscopy: Thudichum speculum — septum, turbinates, polyps
- Look for: DNS, boggy/pale mucosa (allergy), polyps, discharge, bleeding point
- Posterior rhinoscopy: Mirror technique — nasopharynx, adenoids, PND
- Cold spatula test: Fogging pattern to check nasal patency

Throat — Indirect Laryngoscopy (IDL)

- Instrument: Laryngeal mirror (warmed to prevent fogging)
- Position: Patient sitting, tongue protruded, held with gauze
- Visualize: Epiglottis, arytenoids, vocal cords, pyriform fossa
- Assess: Vocal cord mobility (abduction/adduction), any mass/polyp
- Flexible laryngoscopy: Through nose — better view, more comfortable

★ **ENT Exam Order: Ear -> Nose -> Throat!**
Always examine the non-affected side first, then the affected side!

Practice on friends,
practice on patients!
Skills = confidence!

Most Repeated LAQs (10-12 marks)

1. Describe the general examination of a patient in detail
2. Describe the steps of eye examination including visual acuity testing
3. Describe the ear examination including tuning fork tests
4. Write the steps of examination of a patient with dyspnea

Most Repeated SAQs (5-6 marks)

1. Rinne test vs Weber test — interpretation table
2. Visual acuity testing — Snellen chart, recording, pinhole test
3. Pupillary examination — reflexes, RAPD, anisocoria
4. Anterior rhinoscopy — technique and findings
5. Indirect laryngoscopy — instruments and technique
6. Signs of pallor, icterus, cyanosis, clubbing

One-Liner Associations (MCQ/Viva Gold)

Question	Answer
Normal visual acuity	6/6 (Snellen) or 20/20
Pinhole improves vision means	Refractive error (not pathological)
RAPD / Marcus Gunn pupil	Afferent pupillary defect (optic nerve)
Normal IOP range	10-21 mmHg
Gold standard tonometry	Goldmann applanation tonometry
Negative Rinne test means	Conductive hearing loss
Weber lateralizes to normal ear	Sensorineural loss in affected ear
Cone of light position (right ear)	5 o'clock (anteroinferior quadrant)
Thudichum speculum used for	Anterior rhinoscopy
Schamroth sign tests for	Clubbing (loss of nail bed angle)

♥ **Your hands will heal people one day, Sammy!**
Every examination skill you learn now is a life you'll save later!